



THE CLARITY DIAGNOSTIC

747 Reviews. 4,388 Followers. A Marketing Engine That's Been Off Since 2020.

5 findings. The fixes. In order of impact.

My Family Dentist — Downtown Winnipeg | Winnipeg, MB

Prepared by Trent Wehrhahn · H-Cube Marketing · May 2026

I spent some time with My Family Dentist's digital footprint this week. Here's what stood out.

My Family Dentist is one of the strongest under-the-radar practices in downtown Winnipeg. Twelve years under Dr. Esam Beshay's ownership. 747 Google reviews at 4.9 stars — the highest review base in the immediate downtown cluster. A team fluent in four languages. A program mix — First Nations (NIHB), Medavie refugee, Social Assistance, university student offers — that no direct downtown competitor can match in plain language. The clinical and community-facing work is real, and the patients clearly know it.

The reputation has been built. The systems that would turn that reputation into new patient growth haven't.

The gap is between what the practice has earned and what its marketing actually does with it. The blog hasn't published since May 2020. The Instagram has 4,388 followers and 20 lifetime posts — an audience the practice has effectively never spoken to. Booking is phone-only, and the doors are locked from 4pm Monday through 8am the next morning. The website's hero says "You smile .. I smile" — and the meta description Google indexes is eleven keywords with "Winnipeg Dentist" duplicated. None of the things that make this practice distinctive in downtown Winnipeg show up where new patients look first.

4,388

INSTAGRAM FOLLOWERS

Across 20 lifetime posts. The audience exists. Nothing is being said to it.

The five findings below are all fixable. Most are quick. And because the reputation foundation is already there — 747 reviews, 4.9 stars, twelve years of trust — the upside on each fix compounds faster here than at most practices I audit.

How I See This

The thing that strikes me about My Family Dentist is that the hardest part is already done. Most practices we look at are working from a thin review base, an unclear positioning, or a clinical reputation that hasn't translated to the community yet. This practice is the inverse. Dr. Beshay's path — Alexandria University

in 1998, twelve years owning a practice in Egypt, three years rebuilding a license after immigrating to Canada in 2010, buying Dr. Hayes' practice in 2014, growing it to two locations by 2018 — is the kind of owner story that builds long, loyal patient relationships. The team's language access (Punjabi, Arabic, French, English, plus Tigrinya) and the explicit acceptance of NIHB, Medavie refugee, Social Assistance, and post-secondary student insurance programs is a position downtown Winnipeg's other practices generally don't compete on. Patients have responded. Twelve years of clinical work has compounded into 747 reviews at 4.9 stars.

Twelve years. 747 reviews at 4.9 stars. A team fluent in four languages. A position no direct competitor matches. And almost none of it shows up where new patients are looking.

But here's what someone discovering the practice for the first time actually sees. They see a Google Maps panel with strong reviews, but no service-specific category for Invisalign or implants or cosmetic dentistry — so the practice doesn't surface on the searches that high-LTV patients run. They see a homepage tagline that says "You smile .. I smile" instead of telling them this is the practice that takes NIHB, accepts refugee plans, has Arabic and Punjabi-speaking staff, and offers university student rates. They see a contact form and a phone number — and they discover, after they want to book, that there is no online scheduling and the office closes at 4pm. They see an Instagram account with 4,388 followers that hasn't posted in roughly seven years. They see a blog whose most recent post is a COVID safety message from May 2020.

This is what I'd call the Vault With No Door problem. The practice has built real equity. The patients trust it. The community knows it. But the systems that would convert that equity into new patient growth — the GBP categories, the service pages, the online booking, the content cadence, the homepage hero, the meta description — are either misconfigured, dormant, or absent. The five findings below are all variations of the same gap. Fix them in order, and the reputation that's been quietly building for a decade starts working harder than it has ever worked.

The shaw.ca email and the staging URL. Two small things you'll see in Findings 3 and 6 that look minor in isolation, but tell a consistent story: the practice's website and contact infrastructure haven't had an active owner for years. Most fixes are quick. They just need someone to run them.

What makes this tractable is that nothing here requires building from scratch. The practice has reviews, an audience, a domain, a website, social accounts, and a clinically excellent team. The infrastructure exists. What's missing is the system that makes it produce — that captures what the practice is already delivering and converts it into the digital authority a downtown Winnipeg patient researching their next dentist can actually find.

1 747 Reviews at 4.9 Stars. Invisible on the Searches That Pay.

I noticed that My Family Dentist has 747 Google reviews at 4.9 stars — the strongest single review base in the immediate downtown Winnipeg cluster, and roughly 250 reviews above the 500+ standard for an established practice. The website's meta description claims service across the high-margin search categories: Cosmetic Dentist, Teeth Whitening, Dental Implants, Invisalign, Children's Dentist, Emergency Dentist. But on the actual searches, the practice doesn't surface. "Winnipeg Invisalign" is owned by Greenwoods, Cityplace, LifeSmiles, Shine Dental, Reflections, East Kildonan, Southside, Transcona — and not My Family Dentist. "Winnipeg dental implants" is owned by Greenwoods and Dental Designs. The website's homepage icon row lists "Invisalign — invisible aligners," but the navigation menu has no Invisalign service page. The service is mentioned without being substantiated.

MY FAMILY DENTIST

747 reviews 4.9★

VISIBILITY ON "WINNIPEG INVISALIGN"

Not in Maps results

Strong general-search trust signals. Zero presence on the high-margin specialty searches.

WHY THIS MATTERS

\$4,000–\$10,000+

AVERAGE CASE VALUE FOR INVISALIGN OR IMPLANT PATIENTS

Patients searching these terms aren't browsing. They're researching where to spend.

A 747-review base does work on general searches. Someone typing "Winnipeg dentist downtown" or "family dentist downtown Winnipeg" sees the practice's review volume and star rating, and that signal carries weight. But the high-margin specialty searches — Invisalign, implants, cosmetic — operate on different rules. Google ranks practices for those queries based on category signals, not just review volume. The practice has neither a dedicated service page nor (almost certainly) the GBP secondary categories that would make it eligible to rank. So the patient who is researching where to get \$6,000 of

Invisalign treatment finds eight competitors before they find a 4.9-star practice with 747 reviews. The trust signal exists. Google can't tell what the practice does, so it doesn't get to use it.

This is the most expensive gap in the audit because it's where the existing reputation should be doing its hardest work — and isn't.

HERE'S WHAT I'D DO

Add GBP secondary categories (30 minutes): Log into Google Business Profile and add the secondary categories that match the actual service mix: Cosmetic Dentist, Family Dentist, Emergency Dental Service, Children's Dentist. For implants, the closest available secondary category in GBP is typically "Dental Implants Periodontist" — add it if available. There is no native Invisalign category, so Invisalign visibility is driven by service-page content and GBP description keywords (covered below).

Build dedicated service pages for Invisalign, dental implants, and cosmetic dentistry (1–2 weeks each): Each page should be a real piece of content — 800–1,200 words, photos of the actual office, FAQ section addressing cost ranges, before/after gallery if available, and a clear booking CTA. The Invisalign page especially: it's already in the meta description and homepage icon row, but there's nothing for Google to crawl. The dental implants page exists at a thin level — it needs to be expanded with case-type explanations (single tooth, full arch, bone grafting), insurance mention (NIHB and Medavie do cover certain implant cases for eligible patients — that's a unique downtown Winnipeg differentiator), and Dr. Beshay's clinical experience.

Update the GBP description with service-specific keyword density: The description should explicitly name "Invisalign," "dental implants," "cosmetic dentistry," "teeth whitening," "porcelain veneers," "emergency dental," and the practice's accepted insurance programs (NIHB, Medavie, Social Assistance, post-secondary student plans). Naming the programs is doubly useful — it surfaces the practice on searches like "dentist that accepts NIHB Winnipeg" that no direct competitor is targeting.

Effort: Quick win (GBP categories: 30 minutes; 30–60 day indexing movement). Medium for service pages (1–2 weeks each, written once, ranks for years).

2 4,388 Instagram Followers. 20 Posts. Last One Around 2018.

I noticed that My Family Dentist's Instagram (@myfamilydentistca) has 4,388 followers, 120 accounts followed, and 20 lifetime posts. The most recent indexed post URL pattern corresponds to a late 2018 / early 2019 vintage. The website's blog last published in May 2020 — a COVID-era safety message titled "Your Safety is a Top Priority." That's the most recent piece of content the practice has published anywhere on its owned channels. Facebook is semi-active (a Christmas/New Year post in December 2025 was confirmed), but the practice's six-platform social footprint — Facebook, Instagram, Twitter, LinkedIn, Pinterest, YouTube — shows no consistent activity on any of them.

4,388

INSTAGRAM FOLLOWERS

20

INSTAGRAM LIFETIME POSTS

~2018

LAST POST

A captive audience the practice has effectively never spoken to.

An audience of 4,388 people who already followed this practice — and not one new thing has been said to them in roughly seven years.

WHY THIS MATTERS

219:1

INSTAGRAM FOLLOWERS TO LIFETIME POSTS

A captive audience the practice has effectively never spoken to.

Most practices struggle to build the audience. This one already has it. 4,388 Instagram followers is a meaningful local audience — patients, family members, community connections, and prospective patients who've already decided the practice is worth following. The cost of acquiring those followers is sunk. The cost of speaking to them now is essentially zero. But the silence has its own price. A prospective patient who lands on the IG profile to vet the practice sees a dormant account — an unconscious negative trust signal that contradicts the 747 Google reviews. A blog dormant since 2020 means the website earns no organic traffic for any long-tail search beyond the brand name. And the practice's most distinctive stories — the multilingual team, the NIHB and refugee program work, Dr. Beshay's path from Egypt to Canada to two-location ownership — exist nowhere a prospective patient can encounter them.

This isn't a problem of the practice having nothing to say. It's a problem of nobody being given the time to say it.

HERE'S WHAT I'D DO

Restart the blog with practice-specific topics (first post this month): The blog topics that would do the most work for this practice are exactly the topics it has the most credibility to write about: a guide to dental care under NIHB coverage in Manitoba, what refugee patients need to know about Medavie dental

coverage, Invisalign options for university students in downtown Winnipeg, multi-language dental care in Winnipeg. Each post is local, specific, and genuinely useful — and each one supports the GBP and Maps signals being built in Finding 1. One post per month is the minimum viable cadence.

Restart Instagram with a low-overhead format: The practice doesn't need a content studio. It needs a 30-minute monthly photo session in the office and a posting schedule of 4–6 posts per month — a mix of team intros, before/after photos with patient consent, NIHB/refugee-program acceptance reminders, Dr. Beshay or Dr. Farag answering common questions on camera, and community moments. Every post geotagged to Winnipeg, with 5–8 relevant hashtags (#winnipegdentist, #downtownwinnipeg, #nihbdentist, #refugeedental, #manitoba, #invisalignwinnipeg) — distribution to non-followers happens at the hashtag and geotag layer, not just from the existing follower base.

Send one Instagram post and one Facebook post to the existing 4,388 IG followers and the Facebook audience: Before adding new content, post a simple "We're back" message — a short note from Dr. Beshay reintroducing the practice, the team, and what the practice does for the community. That re-engagement post alone gives the dormant audience a reason to start watching the account again. It's the cheapest reactivation campaign available.

Effort: Medium (content cadence + lightweight production system). The audience is already there — what's needed is a pipeline that produces 4–8 posts per month with minimal overhead.

3 Phone-Only Booking. Closed at 4pm. The Door Is Locked When Patients Are Looking.

I noticed that My Family Dentist's confirmed booking path is phone — 204-982-2888 / 204-943-7271. There is no real-time online scheduling platform integrated. The contact page has a callback request form, not a booking flow. Downtown hours are Monday through Friday 8AM–4PM with "Selected Saturdays" 9AM–2PM (the website does not specify which Saturdays). Sunday is closed. There are no evening hours on any weekday. The homepage's primary booking CTA reads, verbatim: "Do you want to make an appointment? Call us Now!"

A patient who finds the practice on a Friday evening, any time on Sunday, or after 4pm any weekday cannot book and cannot reach staff. The only conversion path is a phone call during the same hours they're at work.

WHY THIS MATTERS

O

BOOKING OPTIONS OUTSIDE MON-FRI 8AM-4PM

The practice's discovery-to-booking conversion path is closed for two-thirds of the week.

Downtown Winnipeg has a particular patient demographic. The office towers around 412 Graham Ave hold corporate employees who finish at 5pm, university students at the U of M with class schedules and bus routes, and shift-working community members. Almost none of them are free to call a dental office between 8am and 4pm on a weekday. The practice's 747 reviews are doing the consideration work — patients are finding it, reading the reviews, deciding it looks like the right place. Then they get to the booking step and discover their only option is a phone call during work hours. Many of them move on.

This is not primarily a critique of the practice's operating hours — extending hours has real staffing and personal costs. It's a critique of the absence of online booking, which is a distinct, addressable gap. A patient who can't call during business hours should still be able to confirm an appointment in real time. Right now they can't — and the competition (Greenwoods, Cityplace, Shine Dental, Dental Designs) increasingly can.

HERE'S WHAT I'D DO

Add real-time online booking (1-2 week setup): Jane App, NexHealth, or Dentalcorp's preferred platforms integrate with most Canadian practice management software (ClearDent, Tracker, Dentrix). Pick a platform, connect it to the schedule, and add the booking link to the website header, the GBP "Book" button, the Instagram bio, and the Facebook page. Once it's live, a patient who lands on the website at 9pm on a Sunday can confirm a Tuesday morning appointment without waiting for the front desk to open.

Add the booking link to every patient touchpoint: The website's homepage CTA needs to change from "Call us Now!" to "Book Online or Call Us." The GBP needs the booking button activated. Instagram bio link, Facebook page button, every email footer — all need the booking URL. Each of these is currently a dead end for after-hours patients.

Pilot one extended evening per week as a 90-day test: Adding a single late Tuesday or Thursday (until 6:30 or 7pm) as a pilot — staffed by the existing team on a rotation — would meaningfully expand the availability window. Track new patient bookings during the extended slot for 90 days. If the volume justifies it, make it permanent. If it doesn't, the pilot ends with no commitment.

Effort: Low-medium (online booking integration: 1-2 weeks; hours pilot: operational planning).

The Most Differentiated Position in Downtown Winnipeg — Buried Mid-Page.

I noticed that My Family Dentist explicitly accepts First Nations (NIHB), Social Assistance, Medavie refugee insurance, and post-secondary student insurance plans, with a team fluent in Punjabi, Arabic, French, and English (Tigrinya per practice intake), plus dedicated student offers. This combination is unmatched by any direct downtown Winnipeg competitor I can find — Downtown Dental Group, Upper Fort Dental, and the broader downtown cluster don't lead with NIHB, refugee, or social assistance acceptance. Greenwoods, Cityplace, and LifeSmiles position around Invisalign and cosmetic. Yet on My Family Dentist's homepage, this differentiator appears in the "Quick facts" icon row — below the hero, below the welcome message, below the rotating slideshow — alongside generic items like "parking available." The hero itself says "You smile .. I smile." And the meta description Google indexes is eleven keywords stuffed end-to-end with "Winnipeg Dentist" duplicated.

The single thing that differentiates this practice in downtown Winnipeg's market — accepted insurance programs and language access — is communicated nowhere Google indexes and below the visible homepage scroll on mobile.

WHY THIS MATTERS

11 keywords, 0 sentences

THE META DESCRIPTION GOOGLE SEES

A keyword-stuffed artifact that doesn't tell anyone what this practice actually does.

A homepage hero and a meta description are the two highest-leverage pieces of copy on any practice's website. The hero is the first impression. The meta description is what Google shows in the search result. Both should answer one question for a prospective patient: "Why this practice and not the other one I just looked at?" Right now, both answer it generically. "You smile .. I smile" doesn't tell a refugee patient that the practice accepts Medavie. It doesn't tell an NIHB patient that they're welcome. It doesn't tell an Arabic-speaking parent that the team can communicate with them in their first language. It doesn't tell a U of M student about the student offers. The differentiating evidence is on the page — but not where it does its job.

The cost of this is harder to measure than the booking gap, but it's at least as significant. Every prospective patient who lands on the homepage and doesn't see themselves in the first scroll is a patient who clicks back to the search results. The practice's own community position — the thing that should be making it the obvious choice for huge segments of downtown Winnipeg — isn't on display.

HERE'S WHAT I'D DO

Rewrite the homepage hero this week: Replace "You smile .. I smile" with a hero that names the actual position. Example direction (not final copy): "Downtown Winnipeg's Dentist for Every Family. NIHB, Medavie Refugee, and Social Assistance Plans Welcome. Punjabi, Arabic, French, and English Spoken." The point isn't to write the perfect headline in this audit — it's to make the homepage's first impression match the practice's real identity. One revision cycle, two hours of writing, immediate impact on every visitor.

Rewrite the meta description as one human sentence: Replace the keyword-stuffed list with a single 150–160 character sentence that names the practice's position. Example direction: "Downtown Winnipeg family dentistry — 12 years on Graham Ave, 4.9 stars, multilingual team, accepting NIHB, Medavie, Social Assistance, and student plans." Google indexes this on the next crawl. Click-through rates from search rise on copy that reads like a sentence, not a keyword list.

Surface the program-acceptance content above the fold on the homepage: The "First Nations (NIHB), Social assistance, Medavie (Refugees), Post-secondary Student insurance" content currently lives in an icon row mid-page. Move it into a hero band directly under the headline, with each program named explicitly and clearly. A refugee patient or NIHB patient should see themselves in the first scroll, not after scrolling past the rotating slideshow.

Add an explicit "Insurance & Programs" page to the navigation: A standalone page covering NIHB, Medavie refugee, Social Assistance, post-secondary student insurance, and direct billing — with FAQ-style answers to the questions these patients actually ask ("Do I need to pay anything upfront?", "What if my plan doesn't cover all of it?"). This page becomes a search target for queries like "dentist that accepts NIHB Winnipeg" — queries no direct competitor is going after.

Effort: Quick win (homepage hero + meta description rewrite: 1 day, \$0 ongoing). Medium for the dedicated insurance page (1–2 days; ongoing maintenance minimal).

5 The Small Things That Tell Patients No One Is Watching the Website.

I noticed three small but visible items on the website that, taken together, send a consistent signal: nobody at the practice — or at the agency that built the site — has been actively maintaining it for years. First: the Yoast SEO structured data graph injected into the homepage hardcodes the canonical reference to a staging URL (5993zo.saccloud.online) instead of myfamilydentist.ca. This is in

every JSON-LD @id, url, WebPage, WebSite, and Organization reference — meaning Google's crawler sees mixed canonical signals on every page. Second: the homepage tagline reads "You smile .. I smile" but the schema description reads "You smile .. We smile" — a small inconsistency that's been live for at least 2.5 years. Third: the practice's primary email across every public listing is myfamilydentist@shaw.ca — a free Shaw consumer ISP address — despite the practice owning the myfamilydentist.ca domain.

The schema points to the wrong domain. The tagline doesn't match itself. The email is a free Shaw address. None of these is fatal. Together they tell a consistent story: the website hasn't had an active owner in years.

WHY THIS MATTERS

October 2023

LAST "DATEMODIFIED" IN THE HOMEPAGE SCHEMA

The homepage hasn't been substantively updated in 2.5+ years.

Each of these on its own is small. The schema bug bleeds a fraction of a fraction of ranking strength on every page. The tagline inconsistency is invisible to most readers. The shaw.ca email is a credibility signal at the margin, not a deal-breaker. But for a practice with 747 reviews, two locations, and twelve years of clinical excellence behind it, these are the kinds of small things that should have been fixed years ago. They cumulatively tell a prospective patient — particularly a referring physician or insurance coordinator — that the practice is run by people who do the clinical work brilliantly and haven't gotten to the marketing layer. That's accurate. It's also fixable in a single afternoon.

The deeper signal: a site whose schema URL has been broken for 2.5+ years is a site whose agency relationship has gone passive. Cezar Group built this site. Whatever the original engagement was, the maintenance cadence has stopped. That's the actual finding.

HERE'S WHAT I'D DO

Fix the Yoast schema URL (under one hour): A WordPress developer with Yoast access can update the canonical URL in the Yoast SEO settings to <https://myfamilydentist.ca> and clear the cached

schema. This is a 15-minute change. The benefit: every page on the site stops sending mixed canonical signals to Google.

Fix the tagline inconsistency: Either "You smile .. I smile" or "You smile .. We smile" — pick one and make the homepage and schema match. (My read: neither is doing real work. This is a chance to replace it with the hero rewrite from Finding 4.)

Move email off shaw.ca onto the branded domain (a few hours): Set up Google Workspace or Microsoft 365 with `info@myfamilydentist.ca` (and similar for Pembina South). Forward the existing shaw.ca addresses to the new branded ones for 90 days during the transition. Update every directory listing, the website footer, and the contact page. The cost is roughly \$15–\$20/user/month thereafter. The credibility signal is permanent.

Audit the rest of the technical layer while the developer is in there: Mobile viewport (maximum-scale=1 blocks pinch-zoom — change to allow user zoom), the broken email-protection mailto links, the DOM weight of the homepage (157KB before assets), the LCP score on PageSpeed Insights. None of these is in scope for a Clarity Diagnostic, but the next agency or developer engagement should include a technical audit of the website as a starting point.

Effort: Quick win (schema fix and email move: half a day combined).

Other Things I Noticed

Review responses. With 747 Google reviews at 4.9 stars, the practice has earned an exceptional reputation — but whether the practice is responding to those reviews is a separate signal that affects both Google's perception of GBP activity and prospective patients' read of the practice. A practice that responds to every review (especially personalizing replies with patient names where possible) signals an attentive operation. Without GBP access I can't confirm the current response rate, but the workflow is the same regardless: response within 48 hours, personalized, signed by Dr. Beshay or the office manager.

The 12-year owner story isn't being told. Dr. Beshay's path — Alexandria University 1998, 12 years owning a practice in Egypt, NDEB completion December 2012, license January 2013, buying Dr. Hayes' practice February 2014, full ownership of My Family Dentist January 2018 — is a legitimately compelling story for the kind of patient who values relationship-driven, owner-operator dental care. Right now it's a paragraph on the "About The Dentists" page, written in third person, hidden two clicks from the homepage. That story is a conversion asset for patients who specifically seek independent practice over corporate chains.

The Pembina South cross-promotion gap. The two-location footprint (Downtown + Pembina South) is genuine, but the website treats them as roughly equal locations on the homepage rather than as a cross-referral network. A patient who lands on the Downtown location's content has no clear reason to know the Pembina South location exists, or vice versa. Cross-linking each location's services, hours, and contact prominently on the other's page would compound the SEO authority of both. Outside scope for this audit, but worth noting.

Tigrinya is on the team but not on the website.

Per practice intake, Tigrinya is one of the team's spoken languages — but the website lists Punjabi, Arabic, French, and English only. For Eritrean and Ethiopian patients in Winnipeg's downtown community, Tigrinya-speaking dental care is rare and meaningful. If it's offered, it should be on the homepage and the meta description. Outside the scope of the five primary findings, but it's the kind of small change that opens an entire patient segment.

How I'd Sequence the Fixes

This week

- Log into Google Business Profile and add secondary categories: Cosmetic Dentist, Family Dentist, Emergency Dental Service, Children's Dentist (and Dental Implants Periodontist if available)
- Update the GBP description to name Invisalign, dental implants, cosmetic dentistry, and the program acceptance (NIHB, Medavie, Social Assistance, students)
- Rewrite the homepage hero to name the actual differentiator — accepted programs, language access, downtown family dentistry
- Rewrite the meta description as one human sentence reflecting the practice's real position
- Fix the Yoast schema URL pointing to the staging domain — under one hour for a WordPress developer

Next 30 days

- Add real-time online booking (Jane App or NexHealth) and activate the GBP booking button
- Move email from shaw.ca onto info@myfamilydentist.ca via Google Workspace or Microsoft 365
- Add a dedicated "Insurance & Programs" page to the navigation covering NIHB, Medavie, Social Assistance, student insurance, direct billing
- Publish the first blog post in six years — pick one of: a guide to NIHB dental coverage in Manitoba, what refugee patients need to know about Medavie dental, or Invisalign options at an independent downtown Winnipeg practice
- Send a "We're back" post to the dormant Instagram and Facebook audiences from Dr. Beshay
- Begin a post-appointment review request system — text or email, direct Google link, within 4 hours of each visit

30–90 days

- Build dedicated Invisalign service page (800–1,200 words, FAQ, before/after gallery if available, booking CTA)
- Build dedicated dental implants service page with insurance/coverage detail
- Restart Instagram with 4–6 posts per month — geotagged Winnipeg, hashtagged for local + service
- Pilot one extended evening per week (e.g., Tuesday or Thursday until 6:30pm) as a 90-day operational test
- Monitor Maps rankings for "Winnipeg Invisalign," "Winnipeg dental implants," and "Winnipeg cosmetic dentist" at 60 and 90 days
- Publish second blog post — target topic and keyword chosen based on which Maps search shows the most movement post-GBP update

Ongoing

- Maintain review solicitation cadence — 8–12 new reviews per month is the target velocity
- Respond to every Google review within 48 hours, personalized
- Publish one blog post per month on practice-specific topics (NIHB, Medavie, programs, community, services)
- Maintain Instagram posting at 4–6 posts per month with geotag and hashtag structure
- Publish one GBP post per week mirroring blog or Instagram content
- Audit the website's technical layer (LCP, mobile usability, schema integrity) every six months

The Next Step

Everything in this audit is yours to implement. The fixes are spelled out, in order of impact. Most practices won't get to them — not because they don't matter, but because there's never time. If you'd rather have someone run them for you, the Clarity Audit is the next step. We'll go deeper on the things that need it, prioritize against your specific goals, and give you a 90-day plan you can hand off. No pressure either way — the audit you're holding is the audit, not the appetizer.

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